

**PERFORMANCE WORK STATEMENT (PWS)
CLINICAL RESEARCH ASSOCIATE
BROOKE ARMY MEDICAL CENTER (BAMC)**

**PART 1
GENERAL INFORMATION**

1 General: This is a personal services contract, as defined by Federal Acquisition Regulation (FAR) Subpart 37.104 and is intended to create an employer-employee relationship between the Government and the individual contract Health Care Providers (HCPs) only to the extent necessary for providing the health care services required under the contract. The performance of health care services by individual HCPs under a personal services contract are subject to day-to-day supervision and control by health care facility personnel comparable to that exercised over military and civil service HCPs engaged in comparable health care services. Any personal injury claims alleging negligence by the individual HCPs within the scope of the HCP's performance of the personal services contract shall be processed by Department of Defense (DoD) in the same manner as claims alleging negligence by DoD military or civil service HCPs. The contract does not create an employer-employee relationship between the Government and any corporation, partnership, business association or other party or legal entity with which the HCP may be associated. The authority for this contract is 10 United States Code 1089 and 10 United States Code 1091.

1.1 Description of Services/Introduction: The Contractor shall provide Clinical Research Associate in support of the Department of Radiology at the Brooke Army Medical Center (BAMC) at Joint Base San Antonio, located on Fort Sam Houston, TX. These positions have been designated as Emergency Essential Personnel and shall be required to report to work in accordance with (IAW) Medical Treatment Facility (MTF) policy. All contract personnel shall possess qualifications meeting or exceeding the currently recognized national standards as established by The Joint Commission (TJC), provide the highest consideration to patient dignity, and observe the precepts of the American Hospital Association's "Bill of Rights for Patients." Services provided under this contract shall be as comprehensive as Government-supplied facilities, equipment, and support services permit.

1.2 Background: BAMC, a 425-bed Academic Medical Center, is the DOD's largest in-patient hospital and only Level I Trauma Center. BAMC is staffed by more than 8,000 Soldiers, Airmen, Sailors, Civilians, and contract personnel providing care to wounded Service Members and the San Antonio Community at-large. With an average of 1,500 cases per month, BAMC is the most robust and productive healthcare organization within the Military Health System (MHS). The mission of the Military Healthcare System (MHS) is to enhance our Nation's security by ensuring the nation has a healthy military force capable of providing the full range of military operations, supported by a combat ready healthcare system sustaining the health of DoD beneficiaries. Health care services for eligible MHS beneficiaries are provided at MTFs, such as BAMC, which are located on or near military installations or posts.

Hospitals offer more complex, resource-intensive secondary care (e.g., inpatient care, surgery under anesthesia) and primary care at outpatient clinics inside and outside the hospital. Medical Centers offer tertiary (sophisticated diagnosis/treatment of any ailment), secondary, and primary care, hospital services, preventive services, blood bank, and outpatient services. In addition, Medical Centers offer graduate medical education programs for physicians and other training programs for nursing and ancillary personnel.

1.3 Scope: The Contractor shall provide personal service(s). The Contractor shall provide all personnel, equipment, supplies, facilities, transportation, tools, materials, supervision, and other items necessary to perform services as described in the PWS and associated contract documents, except for those items specified in Part 3 Government Property & Services. The Contractor shall perform to the standards and acceptable quality levels identified in this PWS and associated contract documents.

1.4 Objective(s): To provide expertise to support the successful execution of clinical trials from the protocol concept through the clinical study report, ensures completion of study deliverables, typically works on studies with moderate to high complexity (in terms of design, location, phase, etc.), and proactively identifies and resolves clinical project issues and participates in process improvement initiatives as required. Serves as the research subject matter expert for clinical research trials to primary and associate investigators within San for Brooke Army Medical Center, JBSA-Fort Sam Houston, Texas. Supporting BAMC's mission in providing high quality treatment planning, and high-quality treatments to active-duty soldiers, military retirees, and their dependents. The objective of this contract is to provide qualified contract personnel to fulfill a minimum of 100% of the shifts requested while maintaining a turnover rate that shall not exceed 20% in any 12-month period. The following contract requirements are required at a standard that enables the MTF to maintain a high-level of quality patient care:

(a) Clinical Research Associate in support of Department of Radiology/Radiation Oncology section

1.5 General Information

1.5.1 Quality Control Plan (QCP): This acquisition is for commercial services and will be performed using FAR Part 12. The Contractor's existing quality assurance system shall be utilized in accordance with FAR 12.208.

The Contractor's Proposed QCP shall be submitted to the Contracting Officer (KO) through the Contracting Officer's Representative (COR) for review within 30 calendar days after date of contract award (A.001).

The Government will review and either notify the Contractor in writing of acceptance of the plan or return their comments to the Contractor within seven (7) calendar days. If the Government has provided comments, the Contractor shall then have seven (7) calendar days to submit a Final QCP. After receipt of the Final QCP, the Contractor may receive the KO's acceptance in writing. Any proposed changes to the accepted QCP are required to be

resubmitted for acceptance by the KO no later than seven (7) calendar days prior to the anticipated change and before implementation by the Contractor. The timeline noted above will apply for review and acceptance for proposed changes. At a minimum, the QCP must include and answer the following to be acceptable:

(a) A chart showing the organizational structure and lines of authority, the names, qualifications, duties, responsibilities, and classification of each member of the Contractor's Quality Control Team.

(b) How the Contractor will monitor work to ensure performance complies with all deliverables (*e.g., timelines, deadlines, and goals*).

(c) How the Contractor will monitor work to ensure performance complies with all specifications and requirements of the contract, including the contract's clauses.

(d) How the Contractor will monitor and ensure staff qualifications remain current and valid including Department of Defense (DoD) Contractor Personnel Office (DOCPER) processes/approvals throughout contract performance.

(e) How the Contractor will ensure all keys issued will remain controlled items (Paragraph 1.7 Key Control).

(f) How the Contractor will inventory and track maintenance of all Government Provided Equipment/Materials.

(g) How the Contractor will identify, investigate, and correct any non-conforming performance and prevent similar deficiencies in the future; and

(h) How the Contractor will file and save all Quality Control related documents for the life of the contract plus five (5) years.

1.5.1.1 The Contractor shall inform the COR of issues (or potential issues) that might affect the performance within 24 Hours of identifying those issues. Verbal reports shall be followed up by written reports within seven (7) calendar days (A.002).

1.5.2 Quality Assurance: The Government will evaluate the Contractor's performance under this contract in accordance with the Quality Assurance Surveillance Plan (QASP). This plan is primarily focused on what the Government must do to ensure that the Contractor has performed in accordance with the performance standards. It defines how the performance standards will be applied, the frequency of surveillance, and the acceptable quality levels (performance thresholds).

1.5.3 Recognized Holidays: The following provides information on recognized holidays for the purpose of the PWS. If submittal of any documentation (*e.g., deliverables, submittals, etc.*) deadlines fall on a holiday, the closest workday prior to the holiday will apply as the deadline for submittal.

1.5.3.1 U.S. Holidays: Work shall not be performed on U.S. federally recognized holidays occurring during the normal work week, unless otherwise directed by the KO. When a U.S. federally recognized holiday occurs on a Saturday or a Sunday, the holiday is observed on the preceding Friday or following Monday, respectively. Any holidays that are declared by Presidential Executive Order shall be observed in the same manner as the holidays listed below. Closures of the installation due to inclement weather or other such acts of God shall be handled in the same manner, as well.

New Year's Day, January 1st
Martin Luther King, Jr.'s Birthday, 3rd Monday in January
President's Day, 3rd Monday in February
Memorial Day, Last Monday in May
Juneteenth, June 19th
Independence Day, July 4th
Labor Day, 1st Monday in September
Columbus Day, 2nd Monday in October
Veteran's Day, November 11th
Thanksgiving Day, 4th Thursday in November
Christmas Day, December 25th

1.5.4 Operating Hours: The Government facility office hours, facility operating hours, and the Contractor support hour requirements often coincide, however, they may differ. Please refer to the following. The Chief, Radiation Oncology shall establish the routine hours of the Radiation Oncology Clinic. Services may be scheduled between 6:00 am and 6:00 pm Monday through Friday, excluding legal federal holidays, depending on patient treatment schedules. Normal routine clinic hours are 7:00 a.m. to 4:00 p.m., Monday through Friday. The total regular duty hours worked shall not exceed 40 hours per week. Duty hours include a one-hour lunch period, which is not paid by the government.

1.5.4.1 Government Facility Office Hours: The Government facility office hours are 0700 to 1600, Monday through Friday except for U.S. Holidays identified in paragraph 1.5.3.1 above, or when the Government facility is closed due to local or national emergencies, administrative closings, or similar Government directed facility closings.

1.5.4.2 Government Facility Operating Hours: The Government facility operating hours are from 0001 to 2400, Sunday through Saturday (i.e., 24 hours per day, 7 days per week). The Government facility is open for recognized U.S. Holidays identified in paragraph 1.5.3.1 above, and to essential personnel during national emergencies, administrative closings, or similar Government directed facility closings, unless otherwise communicated by the KO or COR to the Contractor.

1.5.4.3 Contractor Support Hours: The Contractor shall be available to provide support Monday through Friday from 0730-1630 hours daily. The Department shall establish a schedule for contract personnel to perform services on various shifts, which range from 8-10 hours. Each shift shall include 60 minutes for lunch, which may vary depending on the section

and/or specific shift assigned. Duty hours are expected to average 40 hours per week. In order to maintain access to systems, no contract personnel shall work less than five (5) shifts per month. Contract personnel shall not provide services at another institution within eight (8) hours of their scheduled shift. Unplanned or unannounced schedule changes may occur, based on necessity, and in concurrence of the Contractor.

When directed by the Government, contract personnel shall remain on-duty to complete patient treatment, where lack of continuity of care would otherwise jeopardize patient health. This provision is not intended to apply to the time required to complete routine tasks (e.g., completion of paperwork or routine administrative tasks at the end of a shift) which are to be completed as part of the normal work schedule.

During times of low workload, the Department, in concurrence with the COR, may choose to either release contract personnel from a scheduled work shift or re-assign them within the MTF to perform same/similar services that are within their scope of practice.

The Government reserves the right to review and validate hours of service performed by contract personnel by any means necessary, which may include the implementation of sign-in/sign-out procedures, which shall be maintained within the designated area. These attendance logs are for the sole use of the Government to ensure compliance with hours claimed in service of this contract and shall not be provided to the Contractor for any reason or purpose. Under no circumstances shall the Government sign a Contractor's timesheet. With the exception of pager and/or on-call services, only actual hours worked on-site shall be claimed by the Contractor for compensation.

1.5.4.3.1 Scheduled Absences: Contract personnel shall provide a notice of leave to both the Department and COR 5 days prior to any planned absences. The Contractor is not required to provide a back-up to perform services during appropriately scheduled absences. The Contractor shall not invoice for periods when contract personnel are not performing services.

1.5.4.3.2 Unscheduled Absences: In the event that contract personnel are suddenly unable to provide services on an assigned duty day, they shall notify both the Department and COR no later than 8 hours prior to their scheduled shift.

1.5.5 Telework: The Government shall not permit contract personnel to telework in support of this requirement, as on-site performance is necessary to meet the needs within this requirement, with the exception of pager and/or on-call services. In furtherance of Continuity of Operations Planning (COOP), a telework program may be enacted to ensure that the Government's mission-critical operations stay operational during times of national emergency or incidents of national significance. Telework services shall be at no additional cost to the Government.

1.5.6 Place of Performance: The work to be performed under this contract will be performed at BAMC, a government facility located at the Brooke Army Medical Center (BAMC) at Joint

Base San Antonio, located on Fort Sam Houston, TX, and BAMC-associated medical facilities located within a 40-mile radius.

1.6 Security Requirements: The following information is provided on security related matters.

1.6.1 Security Program: The Contractor shall have an active security clearance at the Tier 1 level at proposal submission and shall maintain this level of security for the life of the contract. The Tier 1 Investigation (T1), formerly National Agency Check and Inquiries (NACI), is the basic and minimum investigation required on all new Federal employees and many contractors. It consists of a national agency check with written inquiries and searches of records covering specific areas of an individual's background within the previous five (5) years of submission. These inquiries are sent to current and past employers, schools attended, references, and local law enforcement authorities.

1.6.2 Contract Personnel Security Clearance Requirements: Contract personnel performing services under this contract must have an active T1 prior to the commencement of services and must maintain this level of security for the life of the contract. The Contractor shall coordinate and schedule all security requirements and appointments with the COR before contract personnel can provide services. The COR shall provide the Contractor with all required Security documentation, review all Security documentation submitted by the Contractor, and initiate the request for the contract personnel's security action with the Security Manager. The Contractor is responsible for monitoring the coordination and completion of all Security requirements with the COR.

1.6.3 Installation Access: Access to U.S. installations, buildings and controlled areas is limited to personnel who meet security criteria and are authorized. Failure to submit required information/data and obtain required documentation or clearances will be grounds for denying access to U.S. installations, buildings, and controlled areas. The Contractor shall ensure that any subcontract personnel used in performance of this contract complies with these requirements and that all contract personnel, of both the Contractor and any subcontractor utilized by the Contractor, are made aware of and comply with these requirements.

The Contractor shall be aware of and comply with the requirements associated with Installation Access Control. The Government is not liable for any costs associated with performance delays due solely to a firm's failure to comply with Installation Access Control System (IACS) processing requirements.

The Contractor shall return installation passes to the issuing IACS office when the contract is completed or when a Contractor employee no longer requires access.

1.6.3.1 Installation Pass Log: Reserved

1.6.3.2 Individual Termination or Expiration of Employment: The Contractor shall collect all outstanding installation access passes and Common Access Cards (CACs) the same day

employment of any contract personnel has expired or has been terminated and shall return them to the COR within seven (7) calendar days (A.003).

1.6.3.3 Contract Termination or Cancellation: Upon termination or cancellation of this contract, the Contractor shall collect all outstanding installation access passes and CACs and return them to the COR within seven (7) calendar days (A.004). Contract personnel that have agreed to continue providing services under the succeeding Contractor are exempted.

1.6.3.4 Common Access Cards (CACs): The Contractor shall provide the COR with any necessary information required to obtain a CAC or installation access card within seven (7) calendar days prior to contract personnel commencing services (A.005).

1.6.4 Background Checks: The Contractor shall consult with the COR applicable to the location(s) associated in this PWS, and identify the level of background check necessary, as supported by applicable regulation(s). The Contractor shall coordinate and schedule all background check requirements with the COR before contract personnel can provide services. The COR shall provide the Contractor with all required documentation, review all documentation submitted by the Contractor, and initiate the request for the contract personnel's background check with the Security Manager and Provost Marshals Office (PMO). The Contractor is responsible for monitoring the coordination and completion of all background check requirements with the COR.

1.6.4.1 U.S. Citizen: U.S. Citizenship (copy of birth certificate or naturalization papers required). Contract personnel performing under this contract must be U.S. citizens. In addition, if any contract personnel fail to meet the citizenship requirements stated herein the Contractor shall not hold the government liable for not allowing performance under this contract.

Contract personnel that work with or teach youth under the age of 18 years are subject to U.S background checks. The Requiring Activity Manager will determine the extent of background checks required. Contract personnel may begin performance of the contract after receiving a favorable local background check. The Contractor agrees to replace any contract personnel should results of the background check, if required, be unfavorable as determined by the Requiring Activity Manager. The Contractor's replacement of contract personnel with the unfavorable background check shall be provided within 30 calendar days. The Contractor shall also replace contract personnel whose background check, during their tenure of employment, renders them with an unfavorable local or state background check, as determined by the Requiring Activity Manager. The Contractor shall forward a copy to the KO and COR of each favorable and unfavorable background check within the first seven (7) calendar days of each month, along with the associated contract number and position type (A.006). When the Government awards a non-personal services contract directly to an individual, the failure of the individual to provide a favorable background check would be grounds for termination.

1.6.4.2 Personal Appearance / Work Attire: Contract personnel working under this contract shall maintain a clean, neat, and professional appearance upon entering the MTF (prior to changing into scrubs, if applicable). Contract personnel shall not wear clothing made of see-through material, cutoffs, tank tops, torn/ripped clothing, sleepwear, sandals or clothing that

displays offensive slogans or images. Contract personnel shall not wear athletic or athletic styled clothing (e.g., shorts, jogging suits, sweat suits, leggings, or t-shirts). In all cases, contract personnel clothing shall fit to provide a professional appearance in keeping with the normally accepted standards of dress, and not present an unfavorable appearance upon the Government.

1.6.5 Physical Security: The Contractor shall safeguard all Government equipment, information, and property provided for contract personnel use. At the close of each work period, Government facilities, equipment, and materials shall be secured IAW the Army Physical Security Program (AR 190-13).

1.6.6 Operations Security (OPSEC) Requirements: Contract personnel shall adhere to facility security policies and restrictions. Contract personnel shall immediately report suspicious activities to security personnel. Government issued access badges shall not be worn outside designated facilities where visible to the public.

1.7 Key Control: The Contractor shall establish and implement methods of making sure all keys issued to contract personnel by the Government are not lost or misplaced and are not used by unauthorized persons. NOTE - All references to keys include key cards. No keys issued to contract personnel by the Government shall be duplicated. The Contractor shall develop procedures covering key control that shall be included in the Contractor's QCP as identified in paragraph 1.5.1 above. Such procedures shall include turn in of any issued keys by personnel who no longer require access to locked areas. The Contractor shall immediately report any incidents of lost or duplicate keys to the KO. In the event any keys, other than Master keys, are lost or duplicated the Contractor shall, upon direction of the KO, re-key or replace the affected lock or locks; however, the Government, at its option, may replace the affected lock or locks or perform re-keying. When the replacement of locks or re-keying is performed by the Government, the total cost of re-keying or the replacement of the lock or locks shall be deducted from payment due to the Contractor. In the event a Master key is lost or duplicated, all locks and keys for that system shall be replaced by the Government and the total cost deducted from the payment due to the Contractor. The Contractor shall prohibit the use of Government issued keys by any persons other than the contract personnel. The Contractor shall prohibit the opening of locked areas by contract personnel to permit entrance of persons other than contract personnel engaged in the performance of assigned work in those areas, or personnel authorized entrance by the KO. The Contractor shall establish and implement methods of ensuring that all lock combinations are not revealed to unauthorized persons. The Contractor shall ensure that lock combinations are changed when personnel having access to the combinations no longer have a need to know such combinations. These procedures shall be included in the QCP.

1.8 Post-Award Conference/Periodic Progress Meetings: The Contractor shall attend any post-award conference convened by the contracting activity or contract administration office in accordance with FAR Subpart 42.5. The KO, COR, and other Government personnel, as appropriate, shall meet periodically with the Contractor to review the Contractor's performance. At these meetings, the KO or will apprise the Contractor of how the Government views their performance and the Contractor shall apprise the Government of any problems experienced, if

any. Appropriate action shall be taken to resolve outstanding issues. These meetings shall be at no additional cost to the Government. The Contractor shall not make unplanned visits to the MTF without prior approval from either the KO or COR.

1.9 Contracting Officer's Representative (COR): Refer to Part 2 of this PWS for the definition of a COR. As determined by the KO, a COR will be appointed and identified by letter of designation, a copy of which will be provided to the Contractor by the KO. The designation letter states the responsibilities and limitations of the COR, especially regarding changes in cost or price, estimates, or changes in delivery dates. The COR is not authorized to change any of the terms and conditions of the resulting order. The COR monitors all technical aspects of the contract and assists in contract administration. The COR is not authorized to obligate the Government. If the work is not written in the contract, the COR is not authorized to request new work. The COR is authorized to perform the following functions: assure that the Contractor performs the technical requirements of the contract, perform inspections necessary in connection with contract performance, maintain written and oral communications with the Contractor concerning technical aspects of the contract, issue written interpretations of technical requirements, including Government drawings, designs, specifications, monitor Contractor's performance and notifies both the KO and Contractor of any deficiencies, coordinate availability of Government property, and coordinate site entry of contract personnel.

1.10 Key Personnel: The Contractor shall designate in writing both a Contract Manager and Alternate Contract Manager to the KO and COR at least seven (7) calendar days prior to the commencement of services. The Contract Manager and Alternate Contract Manager will coordinate the implementation of services and be responsible for the performance under this contract. Contract Managers shall have full authority to act on behalf of the Contractor on all contract matters relating to the daily operation of this contract, and the Alternate Contract Manager shall act on behalf of the Contractor should the Contract Manager be unavailable. The Contract Manager and/or Alternate Contract Manager shall be available during the Contractor Support Hours, as identified in paragraph 1.5.4.3. The Contractor shall immediately notify the KO and COR of any changes to the designated Contract Manager and/or Alternate Contract Manager and provide a designated replacement in writing within seven (7) calendar days after said change.

(a) Contract Manager & Alternate Contract Manager: The Contract Manager and/or Alternate Contract Manager shall submit to the COR for review a Profile Application Packet with a Curriculum Vitae (CV) for each contract candidate, prior to beginning services under this contract. Submitted Profile Application Packets with CVs shall detail each contract candidate's years of experience, employment history, education, and associated certifications in relation to this requirement. The COR will then forward the Profile Application Packet to the Department for a final review. The Department will then approve or disapprove the submitted Profile Application Packet within seven (7) calendar days of receipt. No personal interviews shall be conducted by the Government.

(b) Special Qualifications: The Contractor shall ensure that all contract personnel are fully trained and qualified to provide a high level of quality services under this contract,

as required for each position type. When hiring personnel, the Contractor shall keep in mind that the stability and continuity of the workforce is essential and shall maintain an adequate workforce for the uninterrupted performance of all tasks defined within this PWS.

(i) Experience: Contract personnel must have a minimum of 5 years of experience within the last 7 years providing services as a Clinical Research Associate specific to Radiation Therapy Services and protocol management. While providing services under this contract, contract personnel are required to comply with any applicable state laws concerning their profession.

(ii) Education: Bachelor's Degree. Contract personnel must be a graduate of an approved accredited college program. Foreign medical graduates must pass the Educational Commission Foreign Medical Graduate (ECFMG) test, as given by the EFCMG. Contract personnel shall meet all Continuing Medical Education (CME) requirements. CME requirements are the responsibility of the Contractor and shall be obtained at no cost to the Government.

(iii) Certification: It is a condition of this contract that all required certifications in this PWS remain current during the life of the contract. Certifications required under these qualifications are the responsibility of the Contractor and shall be maintained at no cost to the Government.

Contract personnel must have and maintain a current certification in Basic Life Support (BLS) and Advanced Life Support (ALS) from the American Heart Association (AHA).

(iv) Licensure: It is a condition of this contract that all required licenses in this PWS remain current during the life of the contract. While providing services under this contract, contract personnel are required to have and maintain a current, valid, unrestricted license to legally practice in at least one (1) of the 50 states in the United States or the District of Columbia, the Commonwealth of Puerto Rico, or a U.S. Territory. Contract personnel are also required to comply with any applicable state laws concerning their profession. Licenses required under these qualifications are the responsibility of the Contractor and shall be maintained at no cost to the Government.

(v) Professionalism: Contract personnel must exercise a positive, helpful demeanor, display courtesy and tact frequently under stressful conditions and duress at an accelerated pace due to life threatening patient situations.

Contract personnel shall possess sufficient initiative, interpersonal relationship skills and social sensitivity such that he/she can relate constructively to a variety of patients from diverse backgrounds.

Contract personnel shall not bill a patient for services rendered under this contract. Contract personnel shall be prohibited from receiving compensation of any kind for patients treated, procedures performed, or any other actions performed, except under the terms and conditions of this contract, and at the rate specified. Contract personnel shall not, while performing services under this contract, advise, recommend or in any way suggest to persons eligible to receive medical care at Government expense that such persons should receive care from the Contractor or any contract personnel at any place other than the MTF.

(vi) Other: Position requires access to DoD computer systems; therefore, provider shall be a U.S. Citizen or otherwise meet the requirements of AR 25-2.

Contract personnel shall be able to read, write, and speak English well enough to effectively communicate with all patients and other personnel.

The Contractor shall not employ contract personnel to provide services under this contract that are currently employed by the U.S. Government, either as active military or civil service. Such employment would be considered as a conflict of interest, in accordance with DoD 5500.07-R.

1.11 Identification of Contract Personnel: All contract personnel that attend meetings, answer Government telephones, or work in other situations in which their contract status is not obvious to third parties, must identify themselves as contract personnel, to include proper marking of signature blocks in correspondence to avoid creating an impression in the minds of members of the public that they are Government officials. The Contractor shall ensure that all documents or reports produced by contract personnel are suitably marked as Contractor products or that Contractor participation is appropriately disclosed.

1.12 Contractor Identification Badges: The Government MTF will provide identification Badges to contract personnel for specific door access. Contract personnel shall wear the MTF-issued Badges on their outermost garment at all times while providing services at the MTF, unless directed otherwise by protocol (such as in the Operating Room). The Contractor shall collect all ID Badges upon completion of the contract, or termination of employee. A listing of issued ID cards shall be furnished to the KO prior to the contract performance date and updated as needed to reflect the contract personnel changes (A.007).

1.13 Contract Personnel Travel: Reserved

1.14 Other Direct Costs: Reserved

1.15 Data Rights: The Government has unlimited rights to all documents and materials produced under this contract. All documents and materials, to include the source codes of software, produced under this contract shall be Government owned and are the property of the Government with all rights and privileges of ownership and copyright belonging exclusively to the Government. These documents and materials may not be used or sold by the Contractor without written permission from the KO. All materials supplied to the Government shall be the

sole property of the Government and may not be used for any other purpose. This right does not abrogate any other Government rights.

1.16 Protection of Government & Contract Information: Per Public Use Notice of Limitations stated by Defense imagery Management operations Center and contained at www.dimoc.mil/resources/limitations/, the Contractor shall not cite any information (e.g., contract information, pictures, locations, etc.) obtained through this contract on any hard copy or digital marketing tools to include its company website.

1.17 Non-Disclosure Requirements: Performance under this contract may require contract personnel to access data and information that is proprietary to a government agency, another Government Contractor, or of such nature that its dissemination or use other than as specified in this PWS would be adverse to the interests of the Government or others. The Contractor and contract personnel shall not divulge, or release data or information developed or obtained under the performance of this PWS, except to authorized Government personnel or upon written approval of the KO. The Contractor shall not use, disclose, or reproduce proprietary data, which bears a restrictive legend, other than as specified in this PWS. All documentation showing individual names or other personal information shall be controlled and protected under the provisions of the Privacy Act of 1974, Public Law 93-579, 5 United States Code (U.S.C.) Section 552a.

1.18 Non-Disclosure Statements: The Contractor shall provide a signed non-disclosure agreement to the Government within seven (7) days prior to commencement of services under this contract (A.008). Any unauthorized disclosure of information by contract personnel may result in the individual being removed from the contract. Standard non-disclosure statements shall be provided, as required, for personnel who may have access to Government data in the course of their duties.

1.19 Organizational Conflict of Interest: Reserved

1.20 Phase-In / Phase-Out Period: To minimize decreases in productivity and to prevent possible negative impacts on additional services, the Contractor shall have personnel on board, during the 30-day phase-in/phase-out periods. During the phase-in period, the Contractor shall become familiar with performance requirements to commence full performance of services on the contract start date.

1.21 Removal of Contract Personnel: During the performance of this contract, the Government reserves the right to immediately require the temporary removal of any contractor personnel whose actions or impaired state raises reasonable suspicion that clear and present danger of harm exists to either patients, MTF personnel or to themselves. This provision is intended for emergency situations only, and not for the purpose of bringing performance issues or other non-urgent concerns to the attention of the Contractor. Upon removal, the Contractor will be provided notification within 24 hours, and request a formal meeting with the KO, COR, and/or Department to discuss further action. The KO will make the final determination as to whether the temporary removal of any contract personnel should become permanent. The

permanent removal of contract personnel does not relieve the Contractor of any performance requirement.

1.22 Environmental Compliance: Reserved

1.23 Required Training

1.23.1 Anti-Terrorism (AT) Level I Training: All contract personnel, including subcontractor personnel, who are employed under the contract, shall complete AT Level I Training within 30 calendar days after contract start date or effective date of incorporation of this requirement into the contract, whichever applies, or within 30 calendar days after employment of new personnel. The Contractor shall submit certificates of completion (A.009) for each affected contract personnel and subcontractor personnel to the COR (or to the KO, if a COR is not assigned) within seven (7) calendar days after completion of this training by all contract personnel and subcontractor personnel. AT Level I awareness training is available at <https://jkodirect.jten.mil>. Contract personnel shall complete refresher training every 12 months except AOR specific AT Level I Training which shall be completed as required by local AT policies. Verification of the refresher training shall be provided to the COR within seven (7) calendar days after completion of the training (A.009).

If training cannot be conducted on the website, the Contractor shall coordinate with the COR who will contact the local AT Officer at their assigned location for classroom training. Contract personnel completing training in the classroom will receive a certificate or the training attendance roster signed by the AT Officer. Verification of the training shall be provided to the COR within seven (7) calendar days after completion of the training (A.009).

1.23.2 Operations Security (OPSEC) Training: All contract personnel, including subcontract personnel, shall complete Level I OPSEC training Per AR 530-1, within 30 calendar days of employment under this contract (A.010). Verification of the training shall be provided to the COR within seven (7) calendar days after completion of the training. OPSEC Level I Training is available at <http://www.cdse.edu/catalog/elearning/GS130.html>. Contract personnel must complete refresher training every 12 months. Verification of the refresher training shall be provided to the COR within seven (7) calendar days after completion of the training (A.010).

If training cannot be conducted on the website, the Contractor shall coordinate with the COR, who will contact the local OPSEC Officer at their assigned location for classroom training. Contract personnel completing training in the classroom will receive a certificate or the training attendance roster signed by the OPSEC Officer. Verification of the training shall be provided to the COR within seven (7) calendar days after completion of the training (A.010).

1.23.3 iWATCH Training: The Contractor and all associated subcontractors shall brief all contract personnel on the local iWATCH program (training standards provided by the requiring activity AT Officer). This locally developed training will be used to inform contract personnel of the types of behavior to watch for and instruct contract personnel to report suspicious activity to the COR. This training shall be completed within 30 calendar days after the contract is awarded and within 30 calendar days after new contract personnel commence contract

performance, with the results reported to the COR within 30 calendar days after the contract is awarded (A.011). Training can be found on the website:

<http://www.myarmyonesource.com/familyprogramsandservices/iwatchprogram/default.aspx>.

1.23.4 Information Assurance (IA) Training:

1.23.4.1 All contract personnel, including subcontract personnel, requiring access to Government information systems shall complete the DoD IA Cyber Awareness Training before the issuance of network access (A.012). Contract personnel must complete refresher training every 12 months. Verification of the refresher training shall be provided to the COR within seven (7) calendar days after completion of the training (A.012).

1.23.4.2 All contract personnel, including subcontract personnel, working Information Technology (IT)/Information Awareness (IA) functions shall comply with DoD and Army training requirements per IA Training Certification and Workforce Management DoD Directive (DoDD) 8570.01, IA Workforce Improvement Program DoD 8570.01-M, and AR 25-2 within six (6) months of appointment to IA/IT functions. Training can be found on the following website: <https://ia.signal.army.mil/DoDIAA/default.asp>.

1.23.4.3 All contract personnel, including subcontractor personnel, performing services under this contract with access to Government information systems must be registered in the Army Training Certification Tracking System (ATCTS) at commencement of services, the web address is <https://atcts.army.mil> (requires CAC to access).

1.23.5 Protected Health Information (PHI): Contract personnel, including subcontract personnel, performing services under this contract with access to PHI and Government information shall complete "HIPAA & Privacy Act Training" per DoDI 5400.11-R, before the issuance of network access (A.013). Contract personnel must complete refresher training every 12 months. Verification of the refresher training shall be provided to the COR within seven (7) calendar days after completion of the training (A.013). Training is available at <https://jkodirect.jten.mil>.

1.23.6 DHA Employee Safety Course: Contract personnel, including subcontract personnel, performing services under this contract shall complete the "DHA Employee Safety Course" per DoDI 6055.01, within 30 calendar days of employment under this contract (A.014). Verification of the training shall be provided to the COR within seven (7) calendar days after completion of the training. Contract personnel must complete refresher training every 12 months. Verification of the refresher training shall be provided to the COR within seven (7) calendar days after completion of the training (A.014).

1.23.7 Joint Staff Insider Threat Awareness: Contract personnel, including subcontract personnel, performing services under this contract shall complete the "Joint Staff Insider Threat Awareness" per DoDI 1322.31, within 30 calendar days of employment under this contract (A.015). Verification of the training shall be provided to the COR within seven (7) calendar days after completion of the training. Contract personnel must complete refresher

training every 12 months. Verification of the refresher training shall be provided to the COR within seven (7) calendar days after completion of the training (A.015).

1.23.8 New Employee Orientation (NEO): Newly in-processed contract personnel shall be required to complete New Employee Orientation (NEO) training provided by the MTF. Thereafter, all contract personnel shall be required to complete mandatory training to meet competency requirements. Orientation and annual training requirements are a condition of employment. All contract personnel shall also participate in orientation to their work area and maintain a competency file (which is different from a credentials file) at the worksite in accordance with TJC standards.

1.23.9 MHS Genesis: The Government will provide training to contract personnel in MHS Genesis, at no additional expense to the Contractor. When directed by the KO, contract personnel shall attend this training in a paid status as part of the normal services required and billed under the contract. Access to such patient data systems is an "Automated Data Processing Sensitive" position requiring compliance with AR 380-19 and AR 380-67.

1.24 Sexual Assault Prevention & Response Program (SAPR)

1.24.1 The DoD's goal is to provide a culture free of sexual assault through an environment of prevention, education, training, response capability, victim support, reporting procedures, and appropriate accountability that enhances the safety and well-being of all persons covered by the DoD Directive. The DHA is one (1) of the agencies of the DoD, and as such follows the directives, instructions, and guidance of the DoD.

1.24.2 All contract personnel will comply with the Department of Defense Directive (DoDD) 6495.01, SAPR Program. More information is available via the following weblink:
<https://www.esd.whs.mil/Portals/54/Documents/DD/issuances/dodd/649501p.pdf?ver=MIV9x4KYK2qsh38GtGegBw%3d%3d>.

1.25 Reserved

1.26 Reserved

1.27 Government Property (GP) & Services: Refer to Part 3 of this PWS for GP for this contract. For all issued GP for this contract, the Contractor shall submit a Property Management Plan to the KO via the COR within seven (7) calendar days after date of contract award (A.016).

PART 2 DEFINITIONS & ACRONYMS

2 Definitions & Acronyms

2.1 Definitions: Although not inclusive of every term used within this PWS, the following provides a list of definitions used throughout this PWS and commonly used in the acquisition field.

Ancillary Personnel – This includes all HCPs except for Medical Doctors (MD) and Doctors of Osteopath (DO).

Backup Personnel – Pre-approved contract personnel designated to move into the position of the primary contract personnel to perform required services for the duration of his/her absence.

Beneficiaries of the Military Healthcare System – Those individuals entitled to care at the MTF in accordance with AR 40-3.

Clinical Privileges – Authorization by the government to provide specific patient care and treatment services in the organization, within well-defined limits, based on the individual's license, education, training, experience, competence, judgment, and physical and mental health.

Composite Healthcare System (CHCS) – An automated medical information system which will provide integrated support for the functional work centers of inpatient and outpatient care facilities, patient administration, patient appointments and scheduling, nursing, laboratory, pharmacy, radiology, and clinical dietetics.

Continuing Education – Education beyond initial professional preparation relevant to the type of patient care delivered in the organization provides current knowledge relevant to the field of practice and is related to findings from quality assurance activities.

Contracting Officer (KO) – A person with the authority to enter, administer, and/or terminate contracts and make related determinations and findings on behalf of the Government. NOTE - The only individual who can legally bind the Government.

Contracting Officer's Representative (COR) – As defined in DFARS 202.101, means an individual designated and authorized in writing by the Contracting Officer to perform specific technical or administrative functions. DoD Instruction (DoDI) 5000.72, Part II Definitions states the following when defining a COR: "Defined in subpart 202.101 of Reference (f). Any individual delegated responsibilities pursuant to subpart 1.602-2 of Reference (e), regardless of local terminology, must be certified in accordance with this instruction. For example, local terminology can be COR, Contracting Officer's Technical Representative (COTR), technical point of contact, technical representative, alternate COR, administrative COR, assistant COR, line-item manager, task order manager, quality assurance personnel, quality assurance

evaluator, or COR management.” In addition, Army Regulation 70-13, Chapter 2, paragraph 2-2g, states, in part, the following when providing other surveillance support personnel to assist the COR when needed, “...These other surveillance support personnel may serve as on-site representatives of the COR in performance of actual contract surveillance if they meet all COR requirements and have been appointed by the KO as alternate CORs.”

Contractor – Supplier or vendor awarded a contract to provide specific supplies or service to the Government. The term used in this contract refers to the prime.

Contract Personnel – Individuals employed by the Contractor to perform services.

Contractor-acquired Property – Property acquired, fabricated, or otherwise provided by the Contractor for performing a contract and to which the Government has title.

Day – Unless otherwise specified, refers to a calendar day.

Defective Service – Service output that does not meet the standard of performance associated with the PWS.

Deliverable – Anything that can be physically delivered but may include non-manufactured things such as meeting minutes or reports.

Facilities – Building, equipment and supplies necessary for the implementation of service by personnel.

Full-Time Equivalent (FTE) – The Contractor is required to provide 1,920 Regular Hours, which averages 160 Regular Hours per month or 40 Regular Hours per Work Week, for each FTE over a 12-month period. The following FTE determination can be used as a general reference:

1.00 FTE = Averages 40 Regular Hours per Work Week
0.75 FTE = Averages 30 Regular Hours per Work Week
0.50 FTE = Averages 20 Regular Hours per Work Week
0.25 FTE = Averages 10 Regular Hours per Work Week

Government-Furnished Property – As reflected in FAR 52.245-1, Government-furnished Property “means property in the possession of, or directly acquired by, the Government and subsequently furnished to contract personnel for performance of a contract. Government-furnished property includes, but is not limited to, spares and property furnished for repair, maintenance, overhaul, or modification. Government-furnished property also includes Contractor-acquired property if the Contractor-acquired property is a deliverable under a cost contract when accepted by the Government for continued use under the contract.

Government Property – All property owned or leased by the Government. Government property includes both Government-furnished and Contractor-acquired property. Government property includes material, equipment, special tooling, special test equipment, and real property. Government property does not include intellectual property and software.

Holiday Hours – Hours of service provided during Federal Holidays by contract personnel.

Military Health System (MHS) Genesis – An electronic health record for MHS and charting system for HCPs at the MTF. It provides enhanced, secure technology to manage health information; a single health record for service members, veterans, and their families; navigates the MTF's appointment scheduling program, pharmacy, lab, and radiology ordering system and is interlinked with other departments within the MTF.

Military Time – On a 24-hour clock, will read as follows:

8:00 am = 0800
9:10 am = 0910
Noon = 1200
9:10 pm = 2110
Midnight = 2400
12:01 am = 0001

Overtime (OT) – Hours of service provided in excess of 40 Regular Hours within a Work Week by contract personnel.

Property Administrator – An authorized representative of the KO appointed in accordance with agency procedures, responsible for administering the contract requirements and obligations relating to Government property in the possession of contract personnel.

Physical Security – Part of security that is concerned with physical measures designed to safeguard personnel; to prevent unauthorized access to equipment, installations, material, and documents; and to safeguard against espionage, sabotage, damage, and theft.

Privileging Process – Process established by the MTF and AR 40-68 for granting clinical privileges to HCPs.

Quality Assessment & Improvement – Those actions taken by the Government to check services to determine if they meet the requirements of The Joint Commission (TJC), U.S. Army Medical Command, quality assurance and risk management program, and ensure that the contract personnel comply with the terms and conditions of the contract.

Quality Assurance – The various functions, including inspection, performed by the Government to determine whether a Contractor has fulfilled the contract obligations pertaining to quality and quantity.

Quality Assurance Surveillance Plan (QASP) – The key Government-developed surveillance process document and is applied to Performance-Based Service Contracting (PBSC). The QASP is used for managing Contractor performance assessment by ensuring that systematic quality assurance methods validate that Contractor quality control efforts are timely, effective, and are delivering the results specified in the contract or task order. The QASP directly

corresponds to the performance objectives and standards (i.e., quality, quantity, timeliness) specified in the PWS. It provides specific details on how the Government will survey, observe, test, sample, evaluate, and document Contractor performance results to determine if the Contractor has met the required standards for each objective in the PWS. The QASP, with very few if any exceptions, is an internal Government document.

Quality Control – All necessary measures taken by the Contractor to assure that the quality of a product or service shall meet contract requirements.

Regular Hours – Up to 40 hours of service provided within a Work Week by contract personnel. Regular Hours do not include Federal Holidays worked.

Replacement Personnel – Contract personnel selected to fill a position for the duration of the contract period that was previously occupied by another contractor personnel.

Scheduled Absence – A prearranged absence from performance of contractual services by contract personnel. These absences are scheduled/arranged in advance.

Subcontractor – One that enters a contract with a prime Contractor. The Government does not have a contract with the subcontractor.

Subcontract Personnel – Individuals employed by the Subcontractor to perform services.

Tricare – A regionalized, tri-serviced, Contractor-supported, DoD-managed healthcare system.

Unscheduled Absence – Absences from performance of contractual services by contract personnel that are not scheduled/arrange in advance, regardless of whether the absences are caused by illness or for other reasons.

Workday – Hours of service provided in a single day by contract personnel.

Work Week – Sunday through Saturday, unless otherwise specified.

2.2 Acronyms: Although not inclusive of every term used within this PWS, or that may be included in an acquisition, the following provides a list of acronyms commonly used in the acquisition field.

AAHC	Accreditation Association for Ambulatory Health Care
ABEM	American Board of Emergency Medicine
AC	Active Component
ACGME	Accreditation Council for Graduate Medical Education
ACLS	Advance Cardiac Life Support
ADP	Automated Data Processing
AFARS	Army Federal Acquisition Regulation Supplement
AHA	American Heart Association

AI	Associate Investigator
ALMS	Army Learning Management System
ALS	Advanced Life Support
AQL	Acceptable Quality Level
AOR	Area of Responsibility
AR	Army Regulation
AT	Anti-Terrorism
ATCTS	Army Training Certification Tracking System
BAMC	Brooke Army Medical Center
BCG	Bacillus Calmette-Guerin
BCLS	Basic Cardiac Life Support
BI	Background Investigation
BLS	Basic Life Support
CAC	Common Access Card
CDC	Center for Disease Control
CDD	Certificate of Data Disposition
CFI	Center for the Intrepid
CFR	Code of Federal Regulations
CHCS	Composite Healthcare System
CLIN	Contract Line-Item Number
CME	Continuing Medical Education
CMP	Civil Monetary Penalty
CNE	Continuing Nursing Education
CONUS	Continental United States (excludes Alaska & Hawaii)
COOP	Continuity of Operations Planning
COR	Contracting Officer's Representative
COTR	Contracting Officer's Technical Representative
CPDB	Consolidated Personnel Database
CPR	Cardiopulmonary Resuscitation
CTSU	Cancer Trials Support Unit
CV	Curriculum Vitae
DA	Department of the Army
DCCS	Deputy Commander for Clinical Services.
DD-250	Department of Defense Form 250 (Receiving Report)
DEA	Drug Enforcement Administration
DEM	Department of Emergency Medicine
DFARS	Defense Federal Acquisition Regulation Supplement
DHA	Defense Health Agency
DHE	Department of Healthcare Education
DMDC	Defense Manpower Data Center
DOCPER	Department of Defense Contractor Personnel Office
DOD	Department of Defense
DODD	Department of Defense Directive
DODI	Department of Defense Instruction
DOM	Department of Medicine
DSA	Data Sharing Agreement

DAU	Data Use Agreement
ECFMG	Educational Commission for Foreign Medical Graduates
EFT	Electronic Funds Transfer
EIPV	Enhanced Potency Inactivated Poliovirus Vaccine
EKG	Electrocardiogram
FAR	Federal Acquisition Regulation
FBI	Federal Bureau of Investigation
FOIA	Freedom of Information Act
FOUO	For Official Use Only
FPCON	Force Protection Condition
FTC	Federal Trade Commission
FTE	Full-Time Equivalent
GFP	Government Furnished Property
GLP	Good Laboratory Practices
GME	Graduate Medical Education
GOG	Gynecologic Oncology Group
GP	Government Property
HB	Hepatitis Type B
HCP	Health Care Provider
HHS	Health and Human Services
HIPAA	Health Insurance Portability & Accountability Act of 1996
HIV	Human Immunodeficiency Virus
HLO	High Level Objective
IA	Information Awareness
IACO	Installation Access Control Office
IACS	Installation Access Control System
IAW	In Accordance With
ID	Identification
IGCE	Independent Government Cost Estimate
IMD	Information Management Division
IRB	Internal Review Board
ISR	Institute of Surgical Research
IT	Information Technology
IV	Intravenous
JBSA	Joint Base San Antonio
JTR	Joint Travel Regulation
JPAS	Joint Personnel Adjudication System
KO	Contracting Officer
LEIE	List of Excluded Individuals and Entities
LOSS	Lines of Sight Supervision
MD	Medical Doctor
MEDCOM	Medical Command
MEDDAC	Medical Department Activity
MHS	Military Health System
MMR	Measles, Mumps and Rubella
MPICAM	Mission Partner Identity, Credential, and Access Management

MTF	Medical Treatment Facility
NACI	National Agency Check with Inquiries
NACLC	National Agency Credit and Law Check
NCCPA	National Commission on Certification of Physician Assistants
NCI	National Cancer Institute
NCOIC	Noncommissioned Officer in Charge
NEO	New Employee Orientation
OCI	Organizational Conflict of Interest
OCONUS	Outside Continental United States (includes Alaska & Hawaii)
OIC	Officer in Charge
OIG	Office of Inspector General
OMB	Office of Management & Budget
OPSEC	Operations Security
OSHA	Occupational Safety & Health Administration
OT	Overtime
OTSG	Office of the Surgeon General
PA	Property Administrator
PAD	Patient Administration Division
PALS	Pediatric Advance Life Support
PBSC	Performance-Based Service Contracting
PCF	Practitioner's credentials file
PHI	Protected Health Information
PI	Principal Investigator
PIA	Privacy Impact Assessment
PII	Personally Identifiable Information
PIPO	Phase-In/Phase-Out
PL	Public Law
PMO	Provost Marshals Office
POC	Point of Contact
POP	Period of Performance
PPE	Personal Protective Equipment
PRN	Pro Re Nata (Latin for "as the situation demands")
PRS	Performance Requirements Summary
PTE	Part-Time Equivalent
PWS	Performance Work Statement
QA	Quality Assurance
QA&I	Quality Assurance & Improvement
QAP	Quality Assurance Program
QASP	Quality Assurance Surveillance Plan
QC	Quality Control
QCP	Quality Control Program
RA	Requiring Activity
RAMS	Random Antiterrorism Measures
RFP	Request for Proposal
RTOG	Radiation Therapy Oncology Group
SAPR	Sexual Assault and Response Program

SAM	System for Award Management
SCR	Service Contract Report
SF	Standard Form
SOP	Standard Operating Procedure
SORN	System of Records Notice
SOW	Statement of Work
T1	Tier 1 Investigation
TAB	Therapeutic Agents Board
TFMC	Tobacco Free Medical Campus
TE	Technical Exhibit
TJC	The Joint Commission
TTP's	Tactics, Techniques & Procedures
OUM	Unit of Measure
USC	United States Code
USPS	United States Postal Service
WAWF	Wide Area Workflow
WHASC	Wilford Hall Ambulatory Medical Center

PART 3

GOVERNMENT PROPERTY & SERVICES

3 Government Property & Services

3.1 Services: The Government shall provide administrative support required for contract performance (except as designated in the PWS), refuse collection, custodial services, and Government-related telephone service. Personal long-distance calls are not authorized.

3.2 Facilities: The Government will provide use of available MTF facilities, as required for contract performance (except as designated in the contract). Contract personnel shall keep government-furnished work areas in a safe, orderly, and clean condition. Any space utilized by contract personnel in performance of this contract may be used for other purposes during their absence. Items of clothing, personal effects, or equipment may not be able to be secured at all locations. The government will not incur any liability for theft, damage to, or loss of such personal items.

3.3 Utilities: The Government will provide use of available MTF utilities, as required for contract performance (except as designated in the contract). The Government will furnish heat, air conditioning, electricity, water, and sewer services. The Contractor shall instruct contract personnel in utilities conservation practices. The Contractor shall operate under conditions that preclude the waste of utilities, which include turning off the water faucets or valves, light switches, etc. after using the required amount to accomplish tasks requiring the use of the utilities.

3.4 Equipment: The Government will provide use of all available MTF equipment required for contract performance (except as designated in the contract). Contract personnel shall keep government-furnished equipment undamaged and in working condition. Contract personnel shall notify the Government whenever maintenance of equipment is required. The Government will provide telephones, computers, scanners, fax machines, printers.

3.4.1 Personal Protective Equipment (PPE): Contract personnel shall wear special protective clothing (such as scrubs) and shoe covers, when required, which shall be supplied by MTF and utilized in accordance with TJC standards. PPE is to be worn during the performance of services and shall not be worn to and from home and work.

3.5 Special Tooling & Test Equipment: Reserved

3.6 Materials: The Government will provide any available Standard Operating Procedures (SOPs), Local Instructions, and Policies as required for contract performance (except as designated in the contract). Contract personnel shall be responsible for safeguarding all government property provided for use. Neither the Contractor nor any of the contract personnel shall disclose or cause to be disseminated any information concerning the operation of the MTF that could result in or increase the likelihood of the possibility of breach of security or interrupt the continuity of operations or which breach the requirements of the Federal Privacy

Act of 1974. However, the Contractor may be required to provide testimony or disposition in cases of due process action.

3.7 Training: In addition to the Required Training under paragraph 1.23, the Government may elect to provide unique training to contract personnel who are performing services under this contract. If the Government elects to provide such training, the Government will provide such training at no additional expense to the Contractor. When directed by the KO or COR, contract personnel shall attend all such training in a paid status as part of the normal services required and billed under the contract. If determined by the KO, such training may require a performance commitment by contractor personnel, in which the Contractor shall reimburse the government (by means of a reduction in an invoice) if contract personnel fail to satisfy the performance commitment after receiving the specified training. The amount of the reimbursement shall be the prorated cost of the training, calculated based on the total cost of the training and the number of months by which the individual contract personnel failed to complete the performance commitment. The length of the performance commitment shall be 12 months, or until the end of all performance under this contract, whichever occurs first.

3.8 Common Access Cards (CAC): The Government will provide Common Access Cards (CAC) via the Mission Partner Identity, Credential, and Access Management (MPICAM) Portal.

PART 4

CONTRACTOR FURNISHED ITEMS & SERVICES

4 Contractor Furnished Property & Services

4.1 General: The Contractor shall provide all contract personnel necessary to perform services as described in the PWS and associated contract documents, except for those items specified in Part 3.

4.2 Secret Facility Clearance: Reserved

4.3 Materials: Reserved

4.4 Vehicles & Equipment: Reserved

4.5 Background Checks: The Contractor shall provide Background Checks in accordance with this PWS and as determined by the Government.

4.6 Health & Immunization Requirements

4.6.1 All contract personnel performing services under this contract shall receive a general physical examination prior to commencement of services and as a condition of employment. The Contractor shall provide the COR, 10 calendar days before beginning services under this contract, a physical examination certificate (A.017) for all contract personnel who will provide services. The certification shall state the date on which the physical examination was completed and the name of the physician who performed the examination. The physician performing the examination shall sign this certification. For the purposes of this paragraph, a physical examination administered more than 45 days prior to performance of the contract will not be considered adequate. The physical examination shall include the following as a condition of employment:

(a) As determined by the COR, KO, and BAMC Policy, medical clearance for respirator fit testing, using the Respirator Medical Evaluation Questionnaire, BAMC Form 1140.

(b) A history to show that contract personnel have completed a primary series of immunization with tetanus and diphtheria toxoids and that a booster dose is current (within the past 10 years). A test for immunity to the hepatitis (Type B) virus with documentation of the results. A profile shall be established to show immune status to hepatitis. Non-immune Healthcare Workers (lacking anti-HB(c) or anti-HB(s)) shall be required to complete an immunization series with a Hepatitis-B vaccine (e.g., Recombivax, Engerix).

(c) Serologic evidence of immunity to measles and rubella or documentation of immunization with measles, mumps, and rubella (MMR) vaccine using the following guidelines:

(i) Employees born before 1957 without documentation of previous vaccination with MMR should receive one (1) dose.

(ii) Employees born in or after 1957 who have received one (1) dose of MMR previously shall receive one (1) booster dose.

(iii) Employees born in or after 1957 without documentation of any previous vaccination with MMR should receive two (2) doses of vaccine, separated by no less than one (1) month.

(iv) Serologic evidence of immunity to varicella or documented history of illness or immunization.

(v) For personnel who do not have proof of having completed a primary series, completion with enhanced potency inactivated poliovirus vaccine (eIPV) is recommended. Reliable history of spending early childhood and attending elementary schools in the United States since may suffice as clinical history of polio immunization but requires physician documentation in the record of the history and examination.

(vi) Contract personnel shall be screened before employment and annually for tuberculosis by a tuberculin skin test using the Mantoux technique (TINE test is disallowed as a substitute). A skin test result of 10 mm of indurations or more shall be required to have a chest roentgenogram and an evaluation performed. A tuberculin skin test of 10 mm of indurations or more will require documentation providing an assessment of the patient (status of infection- active, inactive; need for preventive treatment or not as determined by age, history of BCG (Bacillus Calmette-Guerin) vaccination; duration of skin test positivity, etc.).

(vii) If all the immunizations, medical clearance, fit testing, and tests set forth in the preceding paragraphs have not been completed, the Contractor shall issue a certificate providing evidence of immunizations and tests that have been completed or started and shall provide a schedule for the completion of unfinished immunizations and lab tests. After the schedule is completed, the Contractor must provide an updated and complete certificate. Failure to complete the immunizations as scheduled may be grounds for the KO, upon the advice of the MTF commander or his clinical staff, to determine that such contract personnel is not an acceptable individual to perform services under this contract.

(viii) The KO, with the explicit case-by-case approval of the MTF commander, is empowered to make exceptions to these requirements, for example, in the event of vaccine shortage or bona fide religious exceptions, but nothing herein shall be construed as an imperative or directive upon the KO to make such exceptions.

4.6.2 All contract personnel performing direct healthcare services under this contract, who experience a parenteral (e.g., needle-stick or cut) or mucous membrane exposure (e.g., splash to the eye or mouth) to blood or bloody body fluids, shall receive prompt treatment. The MTF will evaluate the source of exposure for risk of Hepatitis-B, Hepatitis-C, and Human Immunodeficiency Virus (HIV) and will provide a report of the findings to the Contractor and affected contract personnel. It shall be the Contractor's responsibility to provide appropriate treatment as needed to include Tetanus-Diphtheria booster, Immune Globulin, Hepatitis-B vaccine booster, or Hepatitis-B Immune Globulin. The Contractor shall be responsible for providing the contract personnel with initial testing. If the source of exposure was unknown, positive, or considered an elevated risk for HIV infection, follow-up testing shall be conducted three (3), six (6), and 12 months after exposure. In the event of a confirmed or highly suspected parenteral exposure to HIV, the Contractor shall insure that the contract personnel receive appropriate counseling and are referred immediately to a private infectious disease specialist for consideration of any post exposure prophylaxis (e.g., AZT). The Government may require the Contractor to provide evidence of the status of treatment and testing of the individual contract personnel under the contract.

4.6.3 Contract personnel shall receive the current influenza immunization annually as a condition of employment. Verification of current annual influenza vaccination shall be provided no later than December of each year.

4.6.4 Failure to meet the requirements stated herein, or when test results determine that contract personnel have a contagious disease, the KO may, upon the advice of the MTF commander or his clinical staff, determine that such contract personnel is not an acceptable individual to perform services under this contract.

4.6.5 Contract personnel performing services under this contract shall comply with the health and immunization requirements as stated herein at the time of initial request for clinical privileges and annually thereafter. The expense for all physical examinations to comply with the health requirements shall be borne by the Contractor at no additional cost to the Government.

4.6.6 Compliance with health and immunization requirements contained herein are a condition of employment and not suitable for exemption to policy without commander and KO approval.

4.7 Contractor Reports

4.7.1 Monthly Reports: The contractor shall provide a monthly report the COR within the first seven (7) calendar days of each month that identifies the following (A.018): reporting month in reference, Contract and/or Task Order number, awarded number of FTEs on contract, total number of FTEs filled, total number of FTEs unfilled, total number of FTEs that are currently on-boarding. Monthly Reports shall also include the consolidated status of contract candidate's on-boarding packets (e.g., number of packets submitted, packets pending approval, accepted packets, rejected packets, etc.).

4.7.2 Quarterly Reports: The contractor shall provide a quarterly report (3-months) to the COR within the first seven (7) calendar days of the month immediately following the end of the fiscal quarter (i.e., January, April, July, October) that identifies the following requirements (A.019): each contract personnel's compliance with annual, mandatory and/or unique training, health and immunizations, fit testing, licensure, certifications, and life support training. Quarterly reports shall also include expirations for the applicable items listed above.

4.8 Monthly Invoicing: The contractor shall submit monthly, in arrears, itemized invoices in accordance with DFARS 252.232-7003, for services rendered under this contract. Information regarding WAWF-RA is available on the Internet at <https://wawf.eb.mil/>.

PART 5 SPECIFIC TASKS

5.1 Basic Services: The Contractor shall provide services for Clinical Research Associate, who must perform all duties in such a manner, so as to adhere to TJC Standards. The quality of individual contract personnel shall meet or exceed reasonable standards of professional practice for the health care concerned, as determined by the same authority that governs military medical professionals in the same discipline.

5.2 Tasks: Tasks consist of the following:

5.2.1 Shall be responsible for managing multiple research protocols to include conducting an electronic medical record review and assist with comprehensive evaluation of patients to determine eligibility for clinical trial enrollment.

5.2.2 Shall oversee the processes of subject informed consent process, recruitment, subject visits, source document preparation, and appointment scheduling.

5.2.3 Shall engage in sensitive and emotional conversations with cancer patients and their families regarding issues related to clinical trial participation.

5.2.4 Shall identify and register eligible patients to research subjects through Oncology Patient Enrollment Network (OPEN), a web-based registration system for cancer patient enrollment onto NCI – sponsored clinical trials and is integrated with Cancer Trials Support Unit (CTSU).

5.2.5 Shall contact patients, outside clinicians, and Cancer Registrars throughout the United States to determine disease and survival status.

5.2.6 Shall review patient medical records to obtain information about disease and survival status.

5.2.7 Shall send medical release of information forms to outside institutions to acquire medical treatment information.

5.2.8 Shall prepare, ensure and review all patient study related documents submitted including case report forms, pill dairy, Quality of Life Forms, Urology etc.

5.2.9 Shall administer Neurocognitive tests for NRG Oncology patients, as a certified Neurocognitive Test Examiner.

5.2.10 Shall serve as a liaison between NRG and the Radiation Oncology Service.

5.2.11 Shall review and interpret adverse event reports and notify the Principal Investigator (PI) and Associate Investigator (AI) of all adverse events.

5.2.12 Shall coordinate responses from the PI to accurately submit MedWatch reports to FDA and other applicable regulatory authorities, ensuring submissions are appropriate and timely

5.2.13 Shall provide input and support to maintain appropriate documentation for adverse event safety reporting and monitoring.

5.2.14 Shall prepare for all clinical audits and notify investigators of any concerns identified during audits.

5.2.15 Shall respond to audit findings conducted by NCI, FDA, internal auditors, and external/internal regulatory agencies including BAMC IRB, WHASC IRB, and CTSU.

5.2.16 Shall advise principal and associate investigators on financial management of the clinical trial research program.

5.2.17 Shall assist investigators in developing and managing budgets for all research projects.

5.2.18 Shall adhere to Good Laboratory Practices (GLP) and ensure compliance with all protocol requirements regarding pathology specimen submission, including pathology blocks and slides.

5.2.19 Shall coordinate and collect biospecimens as needed, including processing and shipment of all specimens collected from research subjects.

5.2.20 Shall establish a data collection system for patient tracking and follow up through implementation of a patient database.

5.2.21 Shall monitor and verify research data submitted to NRG Oncology to ensure correctness, consistency, and compliance.

5.2.22 Shall serve as Site Administrator for Data Management for Brooke Army Medical Center and Wilford Hall Ambulatory Surgical Center for CTSU and NCI cooperative group research, as tasked by NRG.

5.2.23 Shall participate in and/or direct research and conduct oncology research, including RTOG and GOG legacy studies and new cooperative group studies.

5.3 Service Contract Reporting : N/A

5.3.1 System for Award Management (SAM) Service Contract Report (SCR): The Contractor shall report ALL contract personnel labor hours (including subcontractor personnel labor hours) required for the performance of services provided under this contract for the SAM via a secure data collection site. The Contractor shall be required to completely fill-in all required data fields through the following web address: www.sam.gov. Reporting inputs will be for the labor executed during the period of performance under each Government Fiscal Year (FY), which runs from October 1st through September 30th. While inputs may be reported any time during the FY, all data shall be reported no later than October 31st of each calendar year. Contractors may direct questions to the help desk by clicking “View Assistance for SAM.gov” which is located at the top of the SAM.gov website. From there, you can select the “Contact Our Service Desk”, which will allow you to contact SAM.gov directly. If the contract period of performance ends prior to September 30th, the Contractor has 30 calendar days from the end date of the contract to complete the SAM SCR requirement.

5.3.2 Steps for Submitting a Service Contract Report (SCR)

- (a) Go to www.sam.gov and log in.
- (b) Select Entity Registrations and then select Service Contract Reporting.
- (c) SAM displays your entities which have service contracts and meet the reporting criteria. Select View by entity to see the service contracts for each entity.
- (d) Next, select Add for the service contract against which you want to create a Service Contract Report. Each service contract which meets the FAR Subpart 4.1703 reporting thresholds is displayed.
- (e) You will be taken to the Complete Service Contract Report page. SAM displays the contract details and allows you to report. You are required to enter the following information:
 - (i) **Total Amount Invoiced:** Total dollar amount invoiced for services performed during the previous Government fiscal year under the contract (this amount should include the prime and any subcontract amount).
 - (ii) **Prime Contractor Hours Expended:** Prime contractor direct labor hours expended on the services performed during the previous Government fiscal year. The amount you enter is automatically divided by 1,920 hours to calculate an FTE, displayed under the Prime Contractor Hours Expended as Prime Contractor FTEs.
- (f) Report any required Tier 1 subcontractor information by selecting the Add Tier 1 Subcontract Information button.
- (g) When you are ready to submit the report, select Submit. This saves your report and returns you to the Select Service Contract page where you can create other SCRs or edit an existing SCR.

5.3.3 Steps for Editing a Submitted Report:

(a) Follow steps 1-4 above. Any previously submitted SCR will have a View/Edit button instead of an Add button.

(b) Select View/Edit. You will be able to view the current SCR, edit available fields, and resubmit the record. You can also delete previously entered information altogether.

(i) Standard: All information provided by the Contractor shall be accurate, complete, and not exceed suspense dates noted in corresponding paragraph above.

PART 6

APPLICABLE PUBLICATIONS

6.1 Applicable Publications (Current Editions): The following publications, manuals, regulations, etc. are mentioned in this PWS and are listed below.

- 6.1.1** Department of Defense Contract Security Classification Specification (DD Form 254)
- 6.1.2** Department of Defense Security Agreement (DD Form 441)
- 6.1.3** National Industrial Security Program Operating Manual (DoD 5220.22-M)
- 6.1.4** Installation Access Control (AE Reg 190-16)
- 6.1.5** The Army Physical Security Program (AR 190-13)
- 6.1.6** Contractor Identification (AE Reg 27-715)
- 6.1.7** Department of Defense Privacy Program (DoD 5400.11-R)
- 6.1.8** Information Assurance Workforce Improvement Program (DoD 8570-M)
- 6.1.9** Information Assurance Training Certification & Workforce Management Directive (DoDD 8570.01)
- 6.1.10** Information Assurance (AR 25-2)
- 6.1.11** DoDI 6055.01 Safety & Occupational Health
- 6.1.12** Policies & Procedures for Property Accountability (AR 735-5)
- 6.1.13** Occupational Safety & Health Administration Public Law 91-596
- 6.1.14** Health Insurance Portability & Accountability Act of 1996, (Public Law 104-191)
- 6.1.15** Joint Commission (JC) Manual (Current Edition)
- 6.1.16** MEDCOM Regulation 600-3, dated 5 Dec 90 Personal General, Off Duty Employment
- 6.1.17** Composition, Mission, and Functions of the Army Medical Department (AR 40-1)
- 6.1.18** Army Medical Treatment Facilities & General Administration (AR 40-2)
- 6.1.19** Medical Services: Medical, Dental, and Veterinary Care (AR 40-3)
- 6.1.20** Army Medical Department Facilities/Activities (AR 40-4)
- 6.1.21** Preventive Medicine (AR 40-5)
- 6.1.22** Medical Facility Management of Sexual Assault and subsequent related AMEDD policy memorandums (AR 40-36)
- 6.1.23** Medical Record Administration & Healthcare Documentation (AR 40-66)
- 6.1.24** Clinical Quality Management (AR 40-68)
- 6.1.25** Standards of Medical Fitness (AR 40-501)
- 6.1.26** Immunizations & Chemoprophylaxis (AR 40-562)
- 6.1.27** The Army Physical Security Program (AR 190-13)
- 6.1.28** The Army Privacy Program (AR 340-21)
- 6.1.29** Army Public Affairs, Public Information (AR 360-5)
- 6.1.30** Department of Army Information Security Program (AR 380-5)
- 6.1.31** Information Systems Security (AR 380-19)
- 6.1.32** Information Systems Security Monitoring (AR 380-53)
- 6.1.33** The Department of the Army Personnel Security Program (AR 380-67)
- 6.1.34** Army Substance Abuse Program (ASAP) (AR 380-85)
- 6.1.35** Army Safety Program (AR 385-10)
- 6.1.36** Accident Investigation & Reporting (AR 385-40)
- 6.1.37** Army Substance Abuse Program (AR 600-85)
- 6.1.38** Preventive Medicine (DA PAM 40-111)

- 6.1.39** Occupational Medical Surveillance Manual MEDCOM (DoD 6055.5-M)
- 6.1.40** Sexual Assault Prevention & Response (SAPR) Program (DoDD 6459.01)
- 6.1.41** Index of Command Administrative Publications (PAM 25-31)
- 6.1.42** Regulation 4500.32-4 Military Standard Transportation & Movement
- 6.1.43** Procedures TG 149 Guidelines for Controlling Occupational Exposure to Hazardous Drugs
- 6.1.44** TG 190 Guide to Managing Exposure to Blood borne Pathogens
- 6.1.45** Joint PUB 1-02, 12 Apr 01 (JP 1-02)
- 6.1.46** DoD Dictionary of Military Terms (www.dtic.mil/doctrine/jel/doddict)
- 6.1.47** BAMC Infection Control Policy & Procedure Guide
- 6.1.48** Common Military Training (DODI 1322.31)
- 6.1.49** Joint Ethics Regulation (DoD 5500.07-R)
- 6.1.50** Radiation Protection Program. Mandatory (SAMMC Memo 40-72)
- 6.1.51** NRC Tele therapy License 42-01368-02.
- 6.1.52** Radiation Therapy Standard Operating Procedures
- 6.1.53** Radiation Protection Program. Mandatory (SAMMC Memo 40-72)
- 6.1.54** Human use of Ionizing Radiation From X-Ray, Radium and Radionuclides (SAMMC Memo 40-32)
- 6.1.55** Management And Control of Diagnostic X-Ray Therapeutic X-Ray and Gamma- Beam Equipment (B-MED 521)

PART 7
ATTACHMENT & TECHNICAL EXHIBIT LISTING

7 Attachment & Technical Exhibit List

7.1 Technical Exhibit A:

- DHA-AI 6000.02 Photography, Videotaping, Filming, Recording, and Imaging of Patients, Visitors and Staff Performance Requirements Summary (PRS)

7.2 Technical Exhibit B: Deliverables Schedule

7.3 Attachment 1: Estimated Workload Data

TECHNICAL EXHIBIT A
PERFORMANCE REQUIREMENTS SUMMARY (PRS)

Performance Objective	Reference	Acceptable Quality Levels (AQLs)	Method Of Surveillance
Comply with Specific Task	PWS Para 5	100% - Zero Deviation from Standard	100% Inspection
Maintain Professional Licenses and Certifications	PWS Para 1.10	100% - Zero Deviation from Standard	100% Inspection
Comply with Health and Immunization Requirements	PWS Para 4.6	100% - Zero Deviation from Standard	100% Inspection
Comply with AT/OPSEC Requirements	PWS Para 1.23	100% - Zero Deviation from Standard	100% Inspection
Provide a High Level of Quality Service	PWS Para 1.10	No more than two (2) substantiated complaints per contractor, within a 12-month period	Customer Feedback
Recruitment and Retention of Qualified Personnel	PWS Para 1.4	Fill rate of 100%, Turnover Rate of 20%	100% Inspection
Provide all Deliverables listed in Technical Exhibit B	PWS Part 7	100% - Zero Deviation from Standard	100% Inspection

**TECHNICAL EXHIBIT B
DELIVERABLES SCHEDULE**

<u>Deliverable</u>	<u>Frequency</u>	<u># Of Copies</u>	<u>Medium/Format</u>	<u>Submit To</u>
A.001 – Quality Control Plan (Paragraph 1.5.1)	Within 30 calendar days after award	One (1) copy to the COR	Electronic delivery via email using Microsoft Office.	To the KO through the COR
A.002 – Report of Potential Issues (Paragraph 1.5.1.1)	Verbal report within 24 hours of identifying issues, followed by a written report within seven (7) calendar days	One (1) copy to the COR	Written report delivered electronically via Microsoft Word	To the KO through the COR
A.003 – Individual Termination or Expiration of Employment (Paragraph 1.6.3.2)	Within seven (7) calendar days of an individual's termination of expiration	One (1) for each affected individual to the COR	Hard Copy	To the COR
A.004 – Contract Termination or Cancellation (Paragraph 1.6.3.3)	Within seven (7) calendar days of contract termination of expiration	One (1) for each affected individual to the COR	Hard Copy	To the COR
A.005 – CAC Info (Paragraph 1.6.3.4)	Within seven (7) calendar days prior to contract personnel's commencement of services	One (1) copy of each to the COR	Electronic delivery via email using Microsoft Office	To the COR
A.006 – Favorable & Unfavorable Background Check (Paragraph 1.6.4.1)	Within the first seven (7) calendar days of each month	One (1) copy to both the KO and COR	Electronic delivery via email using Microsoft Office	To the KO and to the COR
A.007 – Contractor Identification Badges (Paragraph 1.12.2)	Prior to contract performance, and when personnel changes occur	One (1) copy of each to the COR	Electronic delivery via email using Microsoft Office	To the COR

<u>Deliverable</u>	<u>Frequency</u>	<u># Of Copies</u>	<u>Medium/Format</u>	<u>Submit To</u>
A.008 – Signed Non-Disclosure Agreement (Paragraph 1.18)	Within seven (7) days prior to commencement of services	One (1) copy of each to the COR	Electronic delivery via email using Microsoft Office	To the COR
A.009 – AT Level 1 Training Certificates (Paragraph 1.23.1)	Both Initial & refresher within seven (7) calendar days after completion of training	One (1) copy of each to the COR	Electronic delivery via email using Microsoft Office	To the COR
A.010 – OPSEC Training Certificates (Paragraph 1.23.2)	Both Initial & refresher within seven (7) calendar days after completion of training	One (1) copy of each to the COR	Electronic delivery via email using Microsoft Office	To the COR
A.011 – iWATCH Training Completion (Paragraph 1.23.3)	Within 30 calendar days after award, and when new personnel are recruited	One (1) copy to the COR	Electronic delivery via email using Microsoft Office	To the COR
A.012 – DoD IA Cyber Awareness Training Certificates (Paragraph 1.23.4.1)	Initial prior to commencement of services, and Refresher within seven (7) calendar days after completion of training, which is required every 12 months	One (1) copy of each to the COR	Electronic delivery via email using Microsoft Office	To the COR
A.013 – Protected Health Information (Paragraph 1.23.5)	Initial prior to commencement of services, and Refresher within seven (7) calendar days after completion of training, which is required every 12 months	One (1) copy of each to the COR	Electronic delivery via email using Microsoft Office	To the COR
A.014 – Verification of DHA Employee Safety Course	Initial within 30 calendar days of contractor recruitment, and Refresher within seven (7) calendar days after	One (1) copy of each to the COR	Electronic delivery via email using Microsoft Office	To the COR

<u>Deliverable</u>	<u>Frequency</u>	<u># Of Copies</u>	<u>Medium/Format</u>	<u>Submit To</u>
(Paragraph 1.23.6)	completion of training, which is required every 12 months			
A.015 – Verification of Joint Staff Insider Threat Awareness (Paragraph 1.23.7)	Initial within 30 calendar days of contractor recruitment, and Refresher within seven (7) calendar days after completion of training, which is required every 12 months	One (1) copy of each to the COR	Electronic delivery via email using Microsoft Office	To the COR
A.016 – Government Property Management Plan (Paragraph 1.27)	Within seven (7) calendar days after date of contract award	One (1) copy to both the KO and COR	Electronic delivery via email using Microsoft Office	To the KO and to the COR
A.017 – Physical Examination Certificate (Paragraph 4.6.1)	10 calendar days before commencement of services	One (1) copy to the COR	Electronic delivery via email using Microsoft Office	To the COR
A.018 – Contractor Monthly Reports (Paragraph 4.7.1)	Within the first seven (7) calendar days of each month.	One (1) copy to both the KO and COR	Electronic delivery via email using Microsoft Office	To the KO and to the COR
A.019 – Contractor Quarterly Reports (Paragraph 4.7.2)	Within the first seven (7) calendar days of each month that immediately follows each fiscal quarter (Jan, Apr, Jun, Oct).	One (1) copy to both the KO and COR	Electronic delivery via email using Microsoft Office	To the KO and to the COR

**ATTACHMENT 1
ESTIMATED WORKLOAD DATA**

ITEM	NAME	ESTIMATED QUANTITY	
1	Clinical Research Associate	Regular Hours	1,920 x 1) = 1920 Total

TOBACCO FREE MEDICAL CAMPUS (TFMC)

In accordance with Army Regulation 600-63, paragraph 7-3, 14 April 2015; Operations Order 15-48 (Army Medical Command (MEDCOM) Tobacco Free Living – USAMEDCOM), 8 May 2015; and any Operations Order, regulation or other instruction implementing, defining or otherwise addressing the Tobacco Free Medical Campus (TFMC) on any military installation or DoD-controlled location, contract personnel are prohibited from using any tobacco product on or within any TFMC while performing under this contract.

TFMCs are established at each installation or DoD-controlled location and include: (1) any property or nonresidential building that is operated, maintained or assigned to support medical activities, including but not limited to, hospitals, medical laboratories, outpatient clinics (including medical, dental, and veterinary facilities), or aid stations operating for the primary purpose of delivering medical care and services for DOD eligible beneficiaries and /or meeting the mission of the Army Medical Command; (2) all other facilities in which medical activities or administration take place, to include HQ MEDCOM and Defense Health Headquarters; (3) all internal roadways, sidewalks and parking lots; and (4) all sidewalks, parking lots and grounds external but adjacent to the building or related to the migratory corridors surrounding the medical facility. The Contractor shall obtain from the COR any orders, regulations, instructions, or other documents implementing, defining, or otherwise addressing the TFMC for any given installation or DoD controlled location where contract personnel may perform under this contract and shall instruct contract personnel on the TFMC limitations for installations or DoD-controlled locations where they may perform under this contract.

Exclusion from Participation in Federal Health Care Programs (October 2015)

1. The Contractor shall not employ or contract with any individual or entity (hereinafter collectively referred to as “person”) to provide items or services that will be included in invoices submitted to the Government under this contract if such person is listed on the Department of Health and Human Services (HHS) Office of the Inspector General (OIG) List of Excluded Individuals and Entities (LEIE) or the TRICARE Sanctioned Provider List. The Government is legally prohibited from paying for provision of items or services by such persons. The prohibition extends to services beyond direct patient care, such as services of persons in executive or leadership roles and administrative and management services, and whether such services are billed separately. The LEIE may be found at <http://oig.hhs.gov/fraud/exclusions.asp>, and the TRICARE Sanctioned Provider list at <http://www.health.mil/Military-Health-Topics/Access-Cost-Quality-and-Safety/Quality-And-SafetyofHealthcare/Program-Integrity/Sanctioned-Providers>. The LEIE and TRICARE Sanctioned Provider List are hereinafter collectively referred to as “the Lists.”

2. Prior to start of contract performance, the Contractor shall (a) query the Lists to determine whether the name of any person the Contractor employs or contracts with to provide services or items for which payment may be made under this contract appears on the Lists, and (b) certify to the Contracting Officer that the Contractor has queried the Lists and no such names appear on either of the Lists.

3. During performance of the contract, and prior to persons other than those whose names were queried in accordance with paragraph 2, above, (hereinafter “new persons”) providing services or items under the contract, the Contractor shall (a) query the Lists as in paragraph 2, and (b) certify to the Contracting Officer that the names of such new persons do not appear on either of the Lists.
4. The Contractor is advised that during performance of the contract, MTF personnel will perform a recurrent recheck of the names of contract personnel working in the MTF against the Lists, as specified in OTSG/MEDCOM Policy Memo 15-037. The Government will notify the Contractor in the event any contract personnel working in the MTF appear on either of the Lists.
5. Should any person providing items or services under the contract appear on either of the Lists at any time during contract performance, the Contractor shall (a) in cases where the Contractor identified the person, notify the Contracting Officer, and (b) promptly remove that person from the contract.
6. Violation of any aspect of the above paragraphs shall be considered a material breach of the contract and may result in termination of the contract.
7. The Contractor is further advised that, in accordance with Civil Monetary Penalties Law [CMP] (codified at 42 USC § 1320a-7a):
 - a. There are steep civil monetary penalties associated with billing the Government for providing items or services by a person on either of the Lists, and with failing to return to the Government any overpayments received for provision of such items or services.
 - b. Billing under the contract for provision of items or services by a person on either List may also result in exclusion of the person that employs or contracts with such person.
8. HHS OIG has issued a Special Advisory Bulletin on the Effect of Exclusion from Participation in Federal Health Care Programs with additional information on the CMP. The Special Advisory Bulletin may be found at [Special Advisory Bulletin and Other Guidance | Exclusions | Office of Inspector General | U.S. Department of Health and Human Services \(hhs.gov\)](#) .

Personally Identifiable Information, Protected Health Information, and Federal Information Requirements (*Revised 10/27/2020*)

1. General Requirements Overview - Personally Identifiable Information (PII), Protected Health Information (PHI) and Federal Information Laws

This Section addresses the Contractor’s requirements under The Privacy Act of 1974 (Privacy Act), The Freedom of Information Act (FOIA), and The Health Insurance Portability and Accountability Act (HIPAA) as set forth in applicable statutes, implementing regulations and Department of Defense (DoD) issuances. In general, the Contractor shall

comply with the specific requirements set forth in this Section and elsewhere in this Contract. The Contractor shall also comply with requirements relating to records management as described herein.

This Contract incorporates by reference the federal regulations and DoD issuances referred to in this Section. If any authority is amended or replaced, the changed requirement is effective when it is incorporated under contract change procedures. Where a federal regulation and any DoD issuance govern the same subject matter, the Contractor shall first follow the more specific DoD implementation unless the DoD issuance does not address or is unclear on that matter. DoD issuances are available at <http://www.dtic.mil/whs/directives>.

For purposes of this Section, the following definitions apply.

DoD Privacy Act Issuances means the DoD issuances implementing the Privacy Act, which are DoDI 5400.11, DoD Privacy and Civil Liberties Programs, January 29, 2019, and DoDI 5400.11- R, Department of Defense Privacy Program, May 14, 2007.

HIPAA Rules means, collectively, the HIPAA Privacy, Security, Breach, and Enforcement Rules, issued by the U.S. Department of Health and Human Services (HHS) and codified at 45 Code of Federal Regulations (CFR) Part 160 and Part 164, Subpart E (Privacy), Subpart C (Security), Subpart D (Breach) and Part 160, Subparts C-E (Enforcement), as amended. Additional HIPAA rules regarding electronic transactions and code sets (45 CFR Part 162) are not addressed in this Section and are not included in the term HIPAA Rules.

DoD HIPAA Issuances means the DoD issuances implementing the HIPAA Rules in the DoD Military Health System (MHS). These issuances are DoDM 6025.18, "Implementation of the Health Insurance Portability and Accountability Act (HIPAA) Privacy Rule in DoD Health Care Programs," March 13, 2019, DoDI 6025.18, Health Insurance Portability and Accountability Act (HIPAA) Privacy Rule Compliance in DoD Health Care Programs, March 13, 2019, and DoDI 8580.02, Security of Individually Identifiable Health Information in DoD Health Care Programs, August. 12, 2015.

Defense Health Agency (DHA) Privacy Office is the DHA Privacy and Civil Liberties Office. The DHA Privacy Office Chief is the HIPAA Privacy and Security Officer for DHA.

2. Records Management

When creating and maintaining official government records, the Contractor shall comply with all federal requirements established by 44 United States Code (U.S.C.) Chapters 21, 29, 31, 33 and 35, and by 36 CFR, Chapter XII, Subchapter B – Records Management. The Contractor shall also comply with DoD Administrative Instruction No. 15 (DoD AI-15), "OSD Records and Information Management Program" (May 3, 2013) and Records Management requirements outlined in the current TRICARE Operations Manual (TOM).

3. Freedom of Information Act (FOIA)

The Contractor shall comply with the following procedures if it receives a FOIA request and immediately contact the DHA FOIA Officer for evaluation/action:

The Contractor shall inform beneficiaries that DHA FOIA procedures require a written request preferably sent via the National FOIA Portal at: www.FOIA.gov. However, requesters may also submit requests via email at DHA.FOIA@mail.mil; or via postal delivery addressed to the DHA Freedom of Information Service Center, 7700 Arlington Boulevard, Suite 5101, Falls Church, Virginia 22042-5101. All FOIA requests shall describe the desired record as completely as possible to facilitate its retrieval from files and to reduce search fees which may be borne by the requestor. Contract and/or Modification numbers must be included in all FOIA requests seeking DHA procurement records. Although the administrative time limit to grant or deny a request (10 working days after receipt) does not begin until the request is received by DHA, the Contractor shall quickly respond to DHA within 10 working days.

In response to requests received by the Contractor for the release of information, unclassified information, documents, and forms which were previously provided to the public as part of routine services shall continue to be made available in accordance with previously established criteria. All other requests from the public for release of DHA records and, specifically, all requests that reference FOIA shall be immediately forwarded to DHA, ATTENTION: Freedom of Information Officer, for appropriate action. Direct contact, including interim replies, between TRICARE contract personnel and such requestors is not authorized. The Contractor shall process requests by individuals for access to records about themselves in accordance with directions from the DHA Freedom of Information Service Center. If such a requestor specifically makes the request under the Privacy Act or does not make clear whether the request is made under FOIA or the Privacy Act, the Contractor shall process the request in accordance with directions from the DHA Privacy Office. If requestor specifically seeks PHI under HIPAA, the Contractor shall follow paragraph 8.1.6, relating to individual rights of access to PHI.

4. Systems of Records

In order to meet the requirements of the [Privacy Act](#) and the DoD Privacy Act Issuances, the Contractor shall identify to the DHA Contracting Officer (CO) systems of records that are or will be maintained or operated for DHA where records of PII collected from individuals are maintained and specifically retrieved using a personal identifier. Upon identification of such systems to the CO, and prior to the lawful operation of such systems, the Contractor shall coordinate with the [DHA Privacy Office](#) to complete systems of records notices (SORNs) for submission and publication in the *Federal Register* as coordinated by the Defense Privacy, Civil Liberties, and Transparency Division, and as required by the DoD Privacy Act Issuances.

Following proper SORN publication and Government confirmation of Contractor authority to operate the applicable system(s), the Contractor shall also comply with the additional systems of records and SORN guidance, in coordination with the DHA Privacy Office, regarding periodic system review, amendments, alterations, or deletions set forth by the

DoD Privacy Act Issuances, Office of Management and Budget (OMB) Memorandum 99-05, Attachment B, OMB Circular A-130, and Privacy Act of 1974 requirements applicable to contract personnel operating systems of records on behalf of federal agencies. The Contractor shall promptly advise the DHA Privacy Office of changes in systems of records or their use that may require a change in the SORN.

5. Privacy Impact Assessment (PIA)

If DHA data is stored on a Contractor-owned system, a PIA is required from the Contractor.

6. Data Sharing Agreement (DSA)

6.1. (Applies if contract requirements involve the use of DHA data (including PII/PHI, a limited data set, or de-identified data))

The Contractor shall consult with the DHA Privacy Office to determine if the Contractor must obtain a DSA or Data Use Agreement (DUA), when DHA data will be accessed, used, disclosed, or stored, to perform the requirements of this Contract.

The Contractor shall comply with the permitted uses established in a DSA/DUA to prevent the unauthorized use and/or disclosure of any PII/PHI, in accordance with the HIPAA Rules and DoD HIPAA Issuances. Likewise, the Contractor shall comply with the DoD Privacy Act Issuances.

Prior to using any data involving PHI for research purposes, as defined by HIPAA, the Contractor must gain approval from the DHA Privacy Board. Thus, the Contractor shall comply with DHA Privacy Board requests for additional documentation.

To begin the DSA request process, the Contractor shall submit a [DSA Application \(DSAA\)](#) to the DHA Privacy Office. Upon approval, the requestor shall choose to enter one (1) of the following agreements, depending on the data involved:

- DSA for De-Identified Data
- DSA for PHI
- DSA for PII Without PHI
- DUA for Limited Data Set

DSAs executed for contract support will expire after 1 year or at the end of the contract option year, whichever comes first. If the contractual use of DHA data will continue after the DSA expiration date, the Contractor shall submit a DSA Renewal Request template to the Privacy Office; however, if the DSA will not be renewed, the Contractor shall close the DSA by providing a Certificate of Data Disposition (CDD) to the DHA Privacy Office.

6.2. (Applies if contract requirements may include human subject research)

This Contract incorporates by reference the Protection of Human Subject Research clause in the Defense Federal Acquisition Regulation Supplement (DFARS) at 48 CFR 252.235-7004. A separate DFARS provision, 48 CFR 235.072(e), requires that the clause be incorporated in contracts that include or may include research involving human subjects in accordance with 32 CFR 219, DoDI 3216.02, and 10 U.S.C. 980, including research that meets exemption criteria under 32 CFR 219.101(b), the clause applies to solicitations and contracts awarded by any DoD component, regardless of mission or funding Program Element Code. Thus, in the event a Contractor participates in a study or demonstration project or other activity that involves human subject research, then the Contractor shall comply with Protection of Human Subject Research clause. COs may not determine whether an activity is exempt from human subject research requirements. If Contractor activity appears to involve human subject research, then the Contractor shall consult the DHA Privacy Office, which may contact the Research Regulatory Oversight Office in the Office of the Under Secretary of Defense for Personnel and Readiness (OUSD(P&R)).

7. Privacy Act and HIPAA Training

The Contractor shall ensure that its entire staff, including subcontractors and consultants that perform work on this Contract receive training on the Privacy Act, HIPAA, and the federal regulations on confidentiality of substance use disorder patient records, 42 CFR Part 2. Refer to FAR 52.224-3 regarding specific requirements for Privacy Training appropriate to the Contractor's scope of involvement with DHA's PHI and its regulatory responsibilities as either a Covered Entity, or Business Associate.

The Contractor shall ensure all contract personnel and subcontractor personnel supply a certificate of all training completion to the Contracting Officer's Representative (COR) within 30 days of being assigned and on an annual basis based on the trainee's birth month thereafter.

8. HIPAA Business Associate Provisions

8.1. Business Associate – General Provisions

The Contractor shall meet the definition of Business Associate, and DHA meets the definition of a covered entity under the HIPAA Rules and the DoD HIPAA Issuances. Therefore, a Business Associate Agreement (BAA) between the Contractor and DHA is required to comply with the HIPAA Rules and the DoD HIPAA Issuances. The Contractor shall use the DoD BAA, which shall be used by all organizational entities within the DoD, referred to collectively as the "DoD Components", located at, <https://www.health.mil/Military-Health-Topics/Privacy-and-Civil-Liberties/Privacy-Contract-Language/HIPAA-Compliant-Business-Associate-Agreement-for-the-MHS>. b.i. and (3)b.ii.

9. Breach Response

[This paragraph 9 is inoperative, and all references herein to “paragraph 9” shall be deemed to refer to the TOM breach responses provisions if the contract incorporates the TOM by reference.

9.1. Definitions Related to Breach response.

9.1.2. **Breach** means a loss of control, compromise, unauthorized disclosure, unauthorized acquisition, or any similar occurrence where: (1) a person other than an authorized user accesses or potentially accesses PII; or (2) an authorized user accesses or potentially accesses PII for other than authorized purposes. The foregoing definition is based on the definition of breach in DoDM 6025.18. Breaches are classified as either possible or confirmed (see the following definitions) and as either cyber or non-cyber (i.e., involving either electronic PII/PHI or paper/oral PII/PHI).

9.1.3. A **possible breach** is an incident where the possibility of unauthorized access is suspected (or should be suspected) and has not been ruled out. For example, if a laptop containing PII/PHI is lost, and the individual contract personnel does not initially know whether the PII/PHI was encrypted, then the incident must initially be classified as a possible breach, because it is impossible to rule out the possibility of unauthorized access to the PII/PHI. In contrast, that possibility can be ruled out immediately, and a possible breach has not occurred, when misdirected postal mail is returned unopened in its original packaging. However, if the intended recipient informs the individual contract personnel that an expected package has not been received, then a possible breach exists until and unless the unopened package is returned to the individual contract personnel. In determining whether unauthorized access should be suspected, the Contractor shall consider at least the following factors:

- How the event was discovered.
- Did the information stay within the covered entity’s control.
- Was the information actually accessed/viewed; and
- Ability to ensure containment (e.g., recovered, destroyed, or deleted).

9.1.4. A **confirmed breach** is an incident in which it is known that unauthorized access could occur. For example, if a laptop containing PII/PHI is lost and the individual contract personnel knows that the PII/PHI is unencrypted, then the individual contract personnel should classify and report the incident as a confirmed breach, because unauthorized access could occur due to the lack of encryption (the individual contract personnel knows this even without knowing whether or not unauthorized access to the PII/PHI has actually occurred). If the laptop is subsequently recovered and forensic investigation reveals that files containing PII/PHI were never accessed, then the possibility of unauthorized access can be ruled out, and the individual contract personnel should re-classify the incident as a non-breach incident.

9.1.5. An **HHS breach** is an incident that satisfies the definition of breach in Section 164.402 of the HIPAA Breach Rule. The text of the HHS definition states:

Breach means the acquisition, access, use, or disclosure of PHI in a manner not permitted under subpart E of this part [i.e., the HIPAA Privacy Rule] which compromises the security or privacy of the PHI.

HHS breach excludes:

Any unintentional acquisition, access, or use of PHI by a workforce member or person acting under the authority of a DoD covered entity or a business associate, if such acquisition, access, or use was made in good faith and within the scope of authority and does not result in further use or disclosure in a manner not permitted under the HIPAA Privacy Rule.

Any inadvertent disclosure by a person who is authorized to access PHI at a DoD covered entity or business associate to another person authorized to access PHI at the same DoD covered entity or business associate, or organized health care arrangement in which the DoD covered entity participates, and the information received as a result of such disclosure is not further used or disclosed in a manner not permitted under the HIPAA Privacy Rule.

A disclosure of PHI where a DoD covered entity or business associate has a good faith belief that an unauthorized person to whom the disclosure was made would not have reasonably been able to retain such information.

Except as provided in this definition, an acquisition, access, use, or disclosure of PHI in a manner not permitted under this issuance is presumed to be a breach unless the DoD covered entity or business associate, as applicable, demonstrates that there is a low probability that the PHI has been compromised based on a risk assessment of at least the following factors:

The nature and extent of the PHI involved, including the types of identifiers and the likelihood of re-identification.

The unauthorized person who used the PHI or to whom the disclosure was made; whether the PHI was acquired or viewed; and the extent to which the risk to the PHI has been mitigated.

9.1.6. A **cybersecurity incident** is a violation or imminent threat of violation of computer security policies, acceptable use policies, or standard security practices, with respect to electronic PII/PHI. A cybersecurity incident may or may not involve a breach of PII/PHI. For example, a malware infection would be a possible breach if it could cause unauthorized access to PII/PHI. However, if the malware only affects data integrity or availability (not confidentiality), then a non-breach cybersecurity incident has occurred.

9.2. General

9.2.1. The breach response requirements shall be followed for all unauthorized use or disclosure of information regardless of whether the information is PHI or solely PII.

9.2.2. Because DoD defines “breach” to include possible (suspected), as well as actual (confirmed) breaches, the Contractor shall implement these breach response requirements. Immediately upon the Contractor’s discovery of a possible breach. These procedures focus on the first two (2) steps (breach identification and reporting) of a comprehensive breach response program, but also require addressing the remaining steps: containment, mitigation (which includes individual notification), eradication, recovery, and follow-up.

9.2.3. The Contractor shall establish internal processes for conducting the procedures set forth below. These processes shall assign responsibility for investigating, classifying, reporting, and otherwise responding to breaches and cybersecurity incidents. The contractor should consult with the DHA Privacy Office where guidance is needed, such as when the contractor is uncertain whether a discovered breach is the contractor’s responsibility (e.g., if the contractor discovers a breach not caused by the contractor), or how the contractor is to classify an incident (breach vs. non-breach, confirmed vs. possible, cyber vs. non-cyber). Under no circumstances will a contractor delay reporting a confirmed or possible breach to the DHA Privacy Office beyond the 24-hour deadline. In conjunction with its initial investigation, the contractor shall immediately take steps to minimize any impact from the occurrence, proceed with further investigation of any relevant details (such as root causes, vulnerabilities exploited), and initiate further breach response steps.

9.2.4. In the event of a cybersecurity incident not involving a PII/PHI breach, the contractor shall follow applicable DoD cybersecurity and NIST requirements, which include United States-Computer Emergency Readiness Team (US-CERT) reporting (see paragraph 9.3). If at any point a contractor finds that a cybersecurity incident involves a PII/PHI breach (possible or confirmed), the contractor shall immediately initiate the reporting procedures set forth below. The contractor shall also continue to follow any required cybersecurity incident response procedures and other applicable DoD cybersecurity requirements.

9.2.5. Contractors shall require subcontractors who discover a possible breach or cybersecurity incident to initiate the incident response requirements herein by reporting the incident to the contractor immediately after discovery. The time of that report to the contractor shall trigger the contractor’s DHA Privacy Office reporting deadline (24 hours) under paragraph 9.3.2. If a cybersecurity incident is involved, the contractor’s deadline for US-CERT reporting (1 hour) runs from the time the incident is confirmed. The contractor shall require the subcontractor to cooperate as necessary to meet these deadlines, maintain records, and otherwise enable the contractor to complete the breach response requirements herein. Alternatively, the contractor and subcontractor may agree that the subcontractor shall report directly to US-CERT and the DHA Privacy Office, and that the subcontractor shall be responsible for completing the response process, provided that such agreement requires the subcontractor to inform the contractor of the incident and the subsequent response actions.

9.2.6. Contractors shall maintain records of all breach and cybersecurity incident investigations, regardless of the outcome. Investigations identifying unauthorized disclosures must be logged for HIPAA and Privacy Act disclosure accounting purposes, whether individual notification is required under the HIPAA Breach Rule.

9.2.7. Contractors, when acting as HIPAA-covered entities, and not as business associates, are not subject to the breach response requirements herein. However, such contractors are subject to both the HIPAA Breach Rule (applicable to them in their capacity as covered entities) and DoD cybersecurity requirements (applicable to them in their capacity as DoD contractors).

9.3. Reporting Provisions

9.3.1. Immediately upon discovery of a possible or confirmed breach or cybersecurity incident, the contractor shall initiate an investigation. If the incident involves electronic PII/PHI, and if the investigation finds a confirmed breach or cybersecurity incident, the contractor shall report it, within 1 hour of confirmation, to the US-CERT Incident Reporting System at <https://forms.us-cert.gov/report/>, as required by the Department of Homeland Security (DHS).

Note - DHS no longer requires US-CERT reporting of non-cyber breaches or unconfirmed electronic breaches. However, DHS permits US-CERT reporting of unconfirmed cyber-related incidents on a voluntary basis. Thus, if a contractor is uncertain whether a possible cyber-related incident should be treated as confirmed and thus reportable, the contractor may voluntarily report the incident.

Before submission to US-CERT, the contractor shall save a copy of the on-line report. After submitting the report, the contractor shall record the US-CERT incident reporting number, which shall be included in the initial report to the DHA Privacy Office as described in paragraph 9.3.2.

Note - Regardless of whether an incident is confirmed as a breach, the contractor must also investigate whether the incident impacts data integrity or availability of PII/PHI. If such an impact is confirmed, then the incident is reportable to US-CERT as a cybersecurity incident. For guidance on investigating the impact on data integrity and availability, refer to DoD cybersecurity and NIST guidance.

The contractor shall provide any updates to the initial US-CERT report by email to soc@us-cert.gov, with the Reporting Number in the subject line. The contractor shall provide a copy of the initial or updated US-CERT report to the DHA Privacy Office if requested. Contractor questions about US-CERT reporting shall be directed to the DHA Privacy Office, not the US-CERT office.

9.3.2. In addition to US-CERT reporting, the contractor shall report to the DHA Privacy Office by submitting the form specified below within 24 hours of discovery of a breach (possible or confirmed), unless the breach falls within a category that the Privacy Office has determined to be not reportable. This 24-hour period runs from the time of discovery, unlike the 1-hour US-CERT reporting period, which runs from the time a cybersecurity incident is confirmed. Thus, depending on the time needed to confirm, the report to the DHA Privacy Office may be due either before or after the US-CERT report.

The breach report form required within the 24-hour deadline shall be sent by e-mail to: DHA.PrivacyOfficer@mail.mil. The contractor shall also e-mail the report to the CO, the COR, and its usual point of contact at the applicable Program Office. Encryption is not required, because reports and notices shall not contain PII/PHI. If electronic mail is not available, telephone notification is also acceptable (at 703-275-6363), but all notifications and reports delivered telephonically must be confirmed in writing as soon as technically feasible.

Contractors shall prepare the breach reports required within the 24-hour deadline by completing the Breach Reporting Department of Defense Form DD 2959 (Breach of PII Report), available at <https://www.esd.whs.mil/Portals/54/Documents/DD/forms/dd/dd2959.pdf>. For non-cyber incidents without a US-CERT number, the contractor shall assign an internal tracking number and include that number in Box 1.e of the DD Form 2959. The contractor shall coordinate with the DHA Privacy Office for subsequent action, such as beneficiary notification, and mitigation. The contractor must promptly update the DD Form 2959 as current information becomes available.

When a Breach Report Form initially submitted is incomplete or incorrect due to unavailable information, or when significant developments require an update, the Contractor shall submit a revised form or forms promptly after the current information becomes available, stating the updated status and previous report date(s) and showing any revisions or additions in red text. The Contractor shall provide updates to the same parties as required for the initial Breach Report Form.

9.4. Individual Notification Provisions

9.4.1. If the DHA Privacy Office determines that individual notification is required, the Contractor shall provide written notification to beneficiaries affected by the breach as soon as possible, but no later than 10 working days after the breach is discovered and the identities and addresses of the beneficiaries are ascertained. The 10-day period begins once the Contractor can determine the identities (including addresses) of the beneficiaries whose records were impacted. If notification cannot be accomplished within 10 working days, the contractor shall notify the DHA Privacy Office.

9.4.2. The Contractor's proposed notification to be issued to the affected beneficiaries shall be submitted to the DHA Privacy Office for approval. The notification to beneficiaries shall include, at a minimum, the following:

- Specific data elements,
- Basic facts and circumstances,
- Recommended precautions the beneficiary can take,
- Federal Trade Commission (FTC) identity theft hotline information, and
- Any mitigation support services offered, such as credit monitoring.

Contractors shall ensure any envelope containing written notifications to affected individuals are clearly labeled to alert the recipient to the importance of its contents, e.g., "Data Breach Information Enclosed," and that the envelope is marked with the identity of the Contractor and/or subcontractor organization that suffered the breach.

If media notice is required, the contractor will submit a proposed notice and suggested media outlets for the DHA Privacy Office review and approval (which will include coordination with the DHA Communications Division).

9.5. In the event the Contractor is uncertain on how to apply the above requirements, the Contractor shall consult with the CO, who will consult with the Privacy Office as appropriate when determinations on applying the above requirements are needed.

The Contractor shall, at no cost to the government, bear any costs associated with a breach of PII/PHI that the Contractor has caused or is otherwise responsible for addressing.

(END OF PWS)